What decision must leadership make?

HRHS Enterprise AI: From Strategic Decision to Engineering Execution | Fictional Healthcare Technology Advisory Case Study

THE OPPORTUNITY: TWO VALUE LEVERS

$0.45–1.1M per year saved by moving more of $640M supply spend on-contract (71% today, 85% target)

$1.0–3.4M per year, net, recovered by preventing denials ($38M written off; 30% to 50% preventable)

11 + 2 AI use cases proposed; two pilots already live without review, one using patient data

Steady-state estimates (±30%) once live; benefits lag costs. Source: Document 03.

vs

THE CONSTRAINT

92% of FY2027 discretionary IT capacity already committed

6,800 uncommitted hours vs ~14,000 needed for a governed first phase

4,600 specialist hours needed are tied up in two major system upgrades (ERP and EHR)

The decision (Dec 9, 2026, scenario): How should HRHS approach enterprise AI, and what can it actually commit to in FY2027, given that approving a strategy does not create engineering capacity?

What do we know, assume, and need to validate?

Finding	Simulated evidence	Implication	Validation status	Decision consequence
No AI governance	No list of AI in use, no risk review, no testing; the denial-prediction pilot ran 5 months with no success measure	Value and risk cannot be compared across 11 proposals	**High within case**	Minimum governance needed under any option
Unresolved patient-data risk with a vendor	Clinical note-taking pilot: vendor's data-use terms never reviewed; contract auto-renews Jan 31	HIPAA applies; no violation identified, but terms must be verified before renewal	**Confirm by Day 30**	Act now regardless of option
Data readiness is uneven	18% of supply catalog records incomplete; 23% of denials coded "other"	Supply chain usable with remediation; clinical data is not	**Validate Days 30 to 60**	Start in supply chain; contract matching follows data cleanup
Specialist capacity is the bottleneck	6,800 of 85,000 hours free; 4,600 specialist hours committed to the ERP and EHR upgrades	Approval does not create capacity	**Validate Days 15 to 20**	Recommendation holds only if capacity is confirmed

Read with care: every finding is simulated within a fictional case. In a real engagement, each would be checked against seven named validation questions before money or people are committed. Estimates are plus or minus 30%.

Which alternative best serves the organization?

On quantified benefits alone, no option clearly pays back within three years. B's case rests on reusable controls and a real use-case pipeline.

A Targeted, Vendor-Led

+	Fastest value (4 to 6 months); lowest 3-year cost
–	Controls applied product by product; vendor dependency
Capacity	9,000 IT hours (~1,200 specialist)
Cost	$2.55M year 1 | $6.35M 3-yr
Value	**$2.85M–$8.83M benefit** Net −$3.50M to +$2.48M

Weighted score **3.43**

B Governed, Phased

Provisional recommendation

+	Shared controls lower the cost of every later use case
–	~$1.4M more than A over 3 years; needs specialists freed up
Capacity	14,000 IT hours (4,600 specialist)
Cost	$3.75M year 1 | $7.75M 3-yr
Value	**$2.30M–$6.96M benefit (base)** Net −$5.45M to −$0.79M

Weighted score **3.52**

C Platform-First

+	Strongest long-term foundation
–	Value deferred to 2028; exceeds the $4.5M budget
Capacity	30,000 IT hours (~11,000 specialist)
Cost	$7.9M year 1 | $12.3M 3-yr
Value	**Not quantified in the analysis**

Weighted score **2.51**

The recommendation changes if: HRHS has no pipeline beyond the first supply-chain phase, or values speed over control (A wins); specialist capacity cannot be validated (a smaller B, else A); capacity is freed and 15+ use cases are committed (C). All options meet the same mandatory controls; the 0.09-point gap is too small to be decisive. Fast-tracking the denial-prediction pilot under controls raises B's benefit to match A's.

How will leadership know whether execution is working?

Day 0
Authorize
Executive team

Days 15 / 20
Capacity allocated, then validated
CIO/CTO + program lead

Day 30
Governance in place
Steering committee

Day 60
Design and readiness
Steering + CISO

~Day 65
Pilot go/no-go
Risk Board + product owner

Day 90
Scale, revise, or stop
CFO + executive team

Ready to pilot (~Day 65)

- Acceptance criteria approved
- Six required controls in place
- Support runbook and rollback
- Pilot users trained

Business value

- On-contract spend: 71% → 85% target
- Share of AI outputs users accept
- Net denial recovery collected
- Finance-validated, reported separately

When to escalate

- Staff not available: within 1 day
- Checkpoint slipping > 2 weeks
- Cost variance > 10% or $100K
- Patient-data incident: within 24 hours

Proceed: criteria met, controls conform

Pivot: evidence favors Option A

Slow: capacity short, so stagger scope

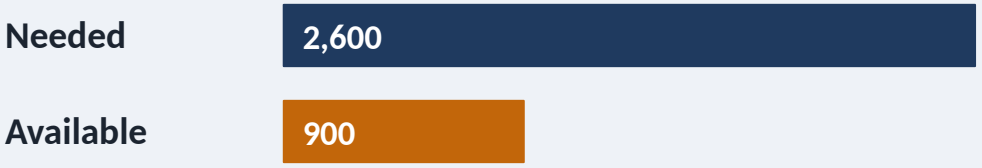
Stop: criteria unmet after one revision

Most plans skip the Day 15 to 20 capacity check. Here is what it caught.

Can the organization actually deliver the decision?

1 THE CONSTRAINT

1,700 specialist hours unavailable when the supply-chain phase needs them (Days 1 to 120)



Hours exist in total (14,000 planned). The right skills at the right time do not: the ERP and EHR upgrades need the same specialists. Found in validation on Day 18 and escalated the same day.

2 THE ALTERNATIVES

1. **Stagger the supply-chain phase**
   Spend classification now, contract matching after ERP go-live: savings arrive ~4 months later

2. **Reallocate engineering capacity**
   Holds the AI schedule, but puts the ERP go-live at risk

3. **Purchase or source externally**
   Adds ~$0.35M and cannot start before ~Day 75

3 THE DECISION: CONFLICT RESOLVED, BOTH GO-LIVES PROTECTED

AI informs. Humans decide. Leaders own.

Stagger the supply-chain phase now: no go-live at risk, savings ~4 months later. Price outside help by Day 60. Move ERP specialists only with written sign-off.